Supplementary Table 2: Group characteristics

No	Author/year Country/ location	Theoretical conceptual underpinning Conceptual approach to community participation (CP).	Initiator of the WG, Reasons for group emerging.	Location where group held. Health service/other Length of time operating	Study design	Outcomes (1) MCH outcomes or (2) in socioemotional wellbeing of mothers and/or children (3) Limitations
1	Lowell, Kildea, Liddle et al (2015)[6] Australia, Remote communities in the Northern Territory (NT)	**Conceptual underpinning** Conceptual base is the importance of incorporating Aboriginal culture in Aboriginal health improvement, two-way learning and community development. **Approach to CP** Developmental and Empowerment	**Initiator** NT Top End Health. To acknowledge the value of cultural knowledge of senior women and recognition of women's skills in tackling their own issues. **Reasons** To begin to reduce Aboriginal disadvantage in maternal and perinatal morbidity and mortality,	**Location** Community based **Length** Commenced in 1993. Evaluation in 1994-5, 2008 and 2015	Qualitative Collaborative participatory evaluation (76 semi structured interviews with program staff (including SW workers), community members and organisations)	**MCH quality care outcomes** Moving towards respect for Aboriginal knowledge in health care and recognition of Aboriginal staff's capacity to improve health outcomes. W**ellbeing outcomes** Benefits included opportunities for employment, recognition of the skills of Aboriginal people in tackling their own issues, and Aboriginal women taught traditional knowledge to their non-Aboriginal partners. **Limitations** Noted the importance of Aboriginal cultural knowledge and practice in birthing was not always shared, and it was difficult to sustain this in some settings when location of control was not with the SWSBSC workers. Two-way learning needed to be stressed. Inadequate funding of the program.
2	Earle-Crane, M. (2000)[24] Canada, Newfoundland, St Johns region	**Conceptual underpinning** Peer support. Effect of disadvantage on pregnancy outcomes **Approach to CP** Instrumental Empowerment	**Initiator** Operationalised through Health Canada Brighter Futures Coalition of St Johns and Daybreak parent Child Centre (possibly NGO). **Reason** Evidence of poorer pregnancy outcomes of childbearing in poverty. Healthy Baby Clubs (HBC) were to target high risk populations and enable positive pregnancy outcomes more effectively than traditional prenatal programs.	**Location** Community-based community centres L**ength** Program operating in Canada since 1994. Report written in 2000.	Qualitative grounded theory (interviews with 20 women attending HBCs)	**Wellbeing outcomes** Sense of comfort, informational and emotional support. Empowerment through new knowledge. Increased self-confidence and giving recognition to unmet needs. Positive outcome on psychological, emotional and social functioning. (Participant perceptions) **Limitations** Author pointed to importance of nursing staff and others understanding the situation of the women so that they can foster the empowerment process.

3	USAID & Burma Shae Thot (2018)[48] Burma, villages and towns in Dry Zone, Yongon and Kayah State	**Conceptual underpinning** Integrated (economic, social, health) community development. Each sector program informed by theory of change. **Approach to CP** Instrumental Developmental Empowerment	**Initiator** USAID/Burma 7-year $70 mil project to provide humanitarian assistance and community development to communities of Central Burma in three areas MCH, WASH and livelihoods and food security. **Reasons** Part of the humanitarian recovery after 2008 cyclone to decrease maternal, newborn and child mortality.	**Location** Community-based **Length** Commenced 2011 (7-year program) evaluated 2018	Mixed methods evaluation Quantitative component was quasi experimental. (4,680 household surveys, 233 village surveys, 23 focus groups, 54 key informant interviews)	**MCH outcomes** Shae Thot program villages showed improvement in prenatal and postnatal, care, nutrition, with increased number of deliveries using clean delivery kits and improved knowledge compared to comparison villages. **MCH service development outcomes** Program established new mobile health services. **Wellbeing outcomes** Village leaders' perceptions of increase in women's leadership. Improvement in knowledge in clean water access and sanitation. **Limitations** The Shae Thot program's 'integrated development approached effective but difficult to separate changes in outcomes driven by Shae Thot's activities from broader trends in Burma's reforms. Shae Thot's achievements did not have equal impact on all communities or members, with some Shae Thot villages reporting low rates of buy-in and participation in community development activities.
4	Saville, N. (on behalf of Mother and Infant Research Activities (MIRA) Kathmandu & Centre for International Health and Development University College London (UCL) (2011)[35] Nepal, Dhanusha, Southern Central region	**Conceptual underpinning** Community mobilisation through participatory action cycles **Approach to CP** Developmental	**Initiator** MIRA a Nepalese non-governmental research organisation established trials in Makwanpur in 2002-3. This trial was conducted in Dhanusha and aimed to improve infant and young child feeding (IYCF). **Reasons** Serious maternal, newborn mortality and poor nutrition.	**Location** Community based non-health services involving government Female Community Health Workers (FCHW) **Length** Mother's groups ran for 2 years.	Quantitative Cluster randomised controlled trial. (over 60,000 births and deaths monitored, 35,000 detailed surveillance questionnaires, 3,300 end point and follow up surveys mother and child pairs)	**MCH outcomes** Cross sectional nutritional study comparing women's group areas with non-women's group areas found there was reduced colostrum discarding, reduced pre-lacteal feeding, more women breast feeding (BF) and fewer feeding another mother's milk or goats milk as first food. Increase in initiation of BF within 1 hour. Women's group areas compared with non-women's group areas showed improvements in complementary feeding – less water, reduced age of initiating tastes of complementary foods and increase in proportion receiving 4 or more food groups.
5	Ministry of Health (Sri Lanka) and UNICEF Sri Lanka (2015)[49]	**Conceptual underpinning** Integrated multi-sector community development framework	**Initiator** MOH Sri Lanka with UNICEF Australian Aid support **Reasons**.	**Location** UNICEF facilitated community-based Mothers Support Groups (MSGs) through the	Mixed methods Evaluation (39 Focus groups with MSGs, 2 Focus	**MCH outcomes** Increased appointments for pregnant women before 8 weeks and proportion of baby health checks before one week. Knowledge of warning signs in pregnancy improved. Proportion of women receiving proper pre-natal care and use of clean delivery kits rose. Child nutrition improved.

	Sri Lanka, Northern and eastern provinces	**Approach to CP** Instrumental	Plateauing of improvements in birth weights and difficulty in reducing the incidence of underweight children and babies **Initiator** MOH Sri Lanka with UNICEF Australian Aid support **Reasons**. Plateauing of improvements in birth weights and difficulty in reducing the incidence of underweight children and babies	government health sector. **Length** Unknown length of time operating. Evaluation conducted in 2014.	groups with MOH office teams, 1 interview with UNICEF program officer, informal feedback from 94 community members)	**MCH service development** Health Clinics were established. **Limitations** The work of the MSG was unknown to some community members.
6	Roy, S. S., Mahapatra, R., Rath, S., Bajpai, A., Singh, V., Rath, S., . . . Prost, A. (2013)[43] India Rural Eastern India Jharkhand and Odisha	**Conceptual underpinning** Concept of community mobilisation to help preventable deaths in the poorest communities. **Approach to CP** Developmental Instrumental	**Initiator** Rese arch group at UCL in association with NGO Ekjut who has helped women's groups to improve maternal and neonatal health in tribal areas of Indian states of Jharkhand and Odisha. **Reason** Poor neonatal and maternal health in the poor tribal areas in India.	**Location** Community based. Eljut NGO women's groups L**ength** Original groups (zone 1) continued to meet to discuss post-neonatal issues. New groups in the original control clusters (zone 2) met to discuss neonatal health. 2005-2011	Quantitative Prospective cohort study plus qualitative interviews (41,191 live births monitored and 41,191 interviews conducted Nov 2004-July 2011)	**MCH outcomes** Data on 41 191 births were analysed. In zone 1, the mean neonatal mortality rate was sustained after the intervention. (34.2 per 1000 live births between 2008 and 2011, 41.3 per 1000 live births between 2005 and 2008). In zone 2: the cluster-mean neonatal mortality rate decreased from 61.8 to 40.5 per 1000 live births between two periods: 2006–2008 and 2009–2011). The intervention resulted in: better hygiene during delivery particularly hand washing and clean cord care, improved thermal care of the neonate with the largest mortality reduction occurring in winter and an increase in exclusive breastfeeding. **Limitations**. It was not possible to disentangle the effect of the women's group intervention on care seeking from the effect of better access to health services. The presence of women's groups alone was not sufficient to increase care-seeking behaviours substantially in the absence of good health service provision. Improvements in home care practices might be strongly driven by the women's group intervention.
7	Gill, K. (1999) [50] Pakistan Urban slum Hyderabad	**Conceptual underpinning** Acceptance that change must come from within with active community participation. Community	**Initiator** Ministry of Health formed partnership supported by other agencies. **Reasons** Problems in the slums with poverty, service access, poor	**Location** Women's groups community based with the urban slum project and have connections with the relevant urban health post.	Mixed methods narrative evaluation (No methodology section)	**MCH outcomes** Showing strong health-related results and quantitative measures available. **MCH service development outcomes** Established innovative health initiative to complement government services. **Wellbeing outcomes**

		development approach, flexible programs, Capacity building, **Approach to CP** Developmental	MCH, lack of infrastructure. Hope was to have overall slum community development.	**Length** Since 1994 Family Welfare urban slum projects morphed into this partnership between NGOs, Ministry, World Bank and numerous other agencies.		Exceptional level of community and NGO participation. Achieved project aims of improving women's status, reaching key target groups. **Limitations** Participatory activities need timely support and resources to succeed. A challenge is defining the role of NGOs and community groups in relation to Government.
8	Ndirangu, G., Gichangi, A., Kanyuuru, L., Otai, J., Mulindi, R., Lynam, P., . . . Archer, L. (2015)[25] Kenya Nairobi urban slums (informal settlements)	**Conceptual underpinning** Mobilizing communities around a health concern was thought to increase awareness of important issues and promote participation and ownership of interventions. **Approach to CP** Instrumental	**Initiator** Jhpeigo an NGO associated with John Hopkins University. **Reasons** To improve participants knowledge about postpartum haemorrhage (PPH), positive behaviour around childbirth, and family planning. Participants lived in poor crowded urban slums with low availability and utilization of formal health services.	**Location** Study conducted at Young Mothers' Clubs (YMCs) in health care facilities. **Length** Since 2005 Jhpeigo NGO has been working with John Hopkins to improve health seeking behaviours	Quantitative quasi experimental (193 pre and post structured interviews completed)	**MCH promotion outcomes** Pre and post quiz on education materials provided to women who attended the YMCs showed that largest improvements in knowledge were about what to include in a birth plan. There was less substantial change in knowledge of danger signs and actions to take in the event of bleeding after delivery.
9	Gram, L., Skordis-Worrall, J., Manandhar, D. S., Strachan, D., Morrison, J., Saville, N., . . . Heys, M. (2018)[36] Nepal Makwanpur	**Conceptual underpinning** A concept of agency as "what the person is free to do and achieve in pursuit of whatever goals or values he or she regards as important". Also, Participatory Learning Action (PLA) **Approach to CP** Original trial Developmental Empowerment	**Initiator** The original trial initiated by MIRA a Nepalese non-governmental research organisation and the UCL. **Reasons** Original trial was to test the participatory women's group approach for improving MCH survival. This trial was to test the long-term impact of a perinatal Participatory Learning Action (PLA) women's group intervention on women's household agency approximately 11.5 years after	**Location** Unclear where groups were located. **Length** Original study conducted on groups running 2001-2003. Follow up study conducted in 2014.	Quantitative quasi experimental (Follow up structured interviews with 4,030 mothers)	**Wellbeing outcomes** This study, assessing impact of original intervention on long term changes in agency at the household level, found no association in the follow-up group between exposure to the original Participatory Learning and Action (PLA) intervention with women's agency in the household approximately 11.5 years later. Some specifications found evidence for reduced agency. Household agency may be a prerequisite for actualising the benefits of PLA groups rather than a consequence. **Limitations** Future work should collect qualitative and quantitative process and implementation data over time to better understand the mechanisms through which women's groups and similar participatory and community-based interventions improve health outcomes.

			individuals original exposure to the intervention.			
10	Roche, M. L., Ambato, L., Sarsoza, J., & Kuhnlein, H. V. (2017[26] Equador Andean Highlands Indigenous Tungurahua Quichua	**Conceptual underpinning** A Positive Deviance/Hearth approach for participatory community nutrition intervention. Health Belief Model. **Approach to CP** Developmental exploring cultural ideas about foods and feeding	**Initiator** World Vision Equador in 2008 **Reasons** In Equador stunting and malnutrition amongst Indigenous people in Andean highlands was serious problem. The objective of the study was to assess the nutritional, social, and cultural potential of mothers' cooking clubs that promoted Quichua culture and traditional foods.	**Location** Meetings were in the Guide mother's home. **Length** The project ran in 2008	Mixed methods study (Interviews and Focus groups with 54 mothers and 16 elders, questionnaires completed by 160 participant and 98 nonparticipant mothers)	**MCH nutrition outcomes**. Dietary diversity scores for the list of promoted feeds were greater for the children 2-5 years of age who had participated in the intervention compared with children from comparison communities. The difference remained significant when controlling for covariates of age socio economic status and maternal education. Intervention mothers fed their children nettle and dock more than the comparison group. **Wellbeing outcomes**. Feeling like a good mother and women and children greeting one another was important (This was usual only between men). Mothers felt stronger with increased self-esteem and pride and security in feeding their children local foods. Increased social network for themselves and for their children and they valued learning the local language.
11	Kruske, S., Schmied, V., Sutton, I., & O'Hare, J. (2004)[51] Australia South East Sydney	**Conceptual underpinning** EBP Adapted from the UK Deptford model. Requires midwives to abandon role of expert. **Approach to CP** Empowerment	**Initiator** Child and Family Health NSW **Reasons** In 1990s Child and Family Health realised that first time mothers were waiting up to three weeks for an appointment for services. Early Bird (EBP) was established to provide confidence and satisfaction for mothers in a timely manner as an inability to provide one-on-one care.	**Location** Community-based government health service **Length** Commenced in 1990s and study published 2004	Qualitative comparative evaluation (20 interviews/ focus groups with EBP women and 20 interviews/ focus groups with non-participants)	**MCH and quality care outcomes** Findings of the study compared Early Bird Program (EBP) attenders with women who did not attend. Women who did not go to the EBP tended to use the child and family health services less. 75% of women who went to the EBP were breast feeding at 8 weeks. **Wellbeing outcomes** 75% of EBP women mentioned that the group support was important. Combination of peer support and professional helped most. EBP attenders self- reported increase in self-esteem and confidence. Learning that most things were normal was important.
12	O'Rourke, K., Howard-Grabman, L., & Seoane, G. (1998)[27] Bolivia Three locations within the remote Bolivian Inquivisi Province	**Conceptual underpinning** Concept of autodiagnosis and participatory community development through women's groups. Paulo Freire, who argued that sustainable social change is only possible if teachers and learners engage in a dialogue,	**Initiator** Funder unclear but probably USAID, Save the Children. Warmi project 1990-1993 commenced in remote area of Bolivia Inquivisi Province **Reasons** Bolivia had one of the highest rates of infant mortality in Latin America. Also had scarce access to modern medical	**Location** Community-based. Unclear whether Bolivian government Remote Health Post was involved. **Length**	Quantitative Quasi experimental (Perinatal mortality rates and obstetric behaviour from 409 women before and after the intervention)	**MCH outcomes** Perinatal and neonatal mortality decreased significantly after the Warmi intervention. **Wellbeing outcomes** Pre and post intervention surveys showed increase in women's awareness of women's groups and increased participation but not related to community, or demographic variables or cultures. Also decrease in participation in some mother's clubs so it was more a redistribution of participation. **Limitations** No clear reasons for increase in attendance or perinatal mortality decrease. Study recommends RCT to better explain

		exchanging ideas and experiences **Approach to CP** Instrumental Developmental Unclear	facilities in rural areas. Following Alma Ata Bolivia tried to involve communities in health care.	The Warmi project ran from 1991-1993.		change and this was a factor leading to the Nepal Manandhar et al (2004) study.
13	Manandhar, D. S., Osrin, D., Shrestha, B. P., Mesko, N., Morrison, J., Tumbahangphe, K. M., . . . Members of the MIRA Makwanpur trial team. (2004)[37] Nepal Makwanpur district	**Conceptual underpinning** MIRA used the Bolivian model, (Warmi) a participatory, rather than a didactic approach, and this might have more effect on perinatal care practices and increase consultation for difficulties in pregnancy and the newborn period. A facilitator rather than teacher necessary. **Approach to CP** Developmental	**Initiator** MIRA a Nepalese non-governmental research organisation supported by DFID (UK Department for International Development) University College London involved. **Reasons** Progress towards the Millennium developmental goals for maternal and child mortality reduction in Nepal had faltered. There were high rates of neonatal and maternal mortality in poor and remote communities.	**Location** Unclear where groups were held. **Length** Trial ran 2001-2003	Quantitative cluster randomised control trial (Birth outcomes of 28,931 women monitored)	**MCH and quality care outcomes** The intervention reduced neonatal mortality by 30%. From 2001 to 2003, the neonatal mortality rate was 26·2 per 1000 (76 deaths per 2899 livebirths) in intervention clusters compared with 36·9 per 1000 (119 deaths per 3226 livebirths) in controls (adjusted odds ratio 0·70 [95% CI 0·53–0·94]). The maternal mortality ratio was 69 per 100 000 (two deaths per 2899 livebirths) in intervention clusters compared with 341 per 100 000 (11 deaths per 3226 livebirths) in control clusters (0·22 [0·05–0·90]). Women in intervention clusters were more likely to have antenatal care, institutional delivery, trained birth attendance, and hygienic care than were controls. **Limitations** Questioned whether the intervention would work where there was no local community health worker and no strengthening of the local health system.
14	Quigley, P., Green, C., Soyoola, M., Kureya, T., Barber, C., & Mubuyaeta, K. (2018)[52] East Africa Rural Zambia (Serenje, Mkushi and Chitambo in Central Province, Mongu in Western Province, and Chama in	**Conceptual underpinning** The programme emphasised inclusive community engagement, empowerment for women and gender equity as key action for improving women's, children's and adolescents' health. **Approach to CP** Instrumental (contribution of men's	**Initiator** The Zambian government and a development company established the More Mobilising Access to Maternal Health Services in Zambia (MORE MAMaZ). **Reasons** Attempt to meet the Millennium goals 2000-2015 for better access to prenatal care and skilled birth attendance (SBA) in poor rural communities.	**Location** Common area in community used for discussions. **Length** Intervention between 2014-2016	Quantitative Quasi experimental with qualitative review (survey data from 3,538 men and women)	**MCH outcomes** Survey and qualitative reviews showed significant difference in intervention communities in antenatal care attendance, skilled care attendance, use of maternal dangers signs and modern family planning knowledge. **Wellbeing Outcomes** Self -reported data showed a substantial effect on the empowerment of women and a reduction in wife beating in intervention communities.

	Muchinga Provincebia)	time to provide transport)				
15	Skordis, J., Pace, N., Vera-Hernandez, M., Rasul, I., Fitzsimons, E., Osrin, D., . . . Costello, A. (2019)[38] Nepal Makwanpur district	**Conceptual underpinning** Extensive conceptual base. Models of health production and health care demand and structure and function of family and community networks as source of private transfers and risk sharing **Approach to CP** Developmental Instrumental	**Initiator** Originally it was MIRA a Nepalese non-governmental research organisation. This study by University College London predominantly. **Reasons** To better understand the decision-making processes behind care-seeking and the implications of family networks in rural Nepal.	**Location** The original women's groups in the 2002-2003 trial were community-based. **Length** The original groups 2001-2003	Quantitative cross sectional. This is a later study than the 2002-3 Makwanpur trial but using these data. No groups established for this study.Trial questionnaire and social network questionnaire from 1749 women)	**MCH promotion outcomes** More frequent PWG participation results in increased health knowledge. Less poor women have greater knowledge. Family networks do not affect health knowledge. Estimates of good MCH practice by women show that level of knowledge is a predictor. Number of husband's relatives in a woman's family network negatively and significantly predicts care practice.
16	Tripathy, P., Nair, N., Barnett, S., Mahapatra, R., Borghi, J., Rath, S., . . . Costello, A. (2010)[44] India Three districts in East India, largely tribal and rural	**Conceptual underpinning** Theory about participatory learning, and capacity building through decision-making **Approach to CP** Developmental empowerment (as in Makwanpur)	**Initiator** The study group initiated the controlled trial, funded externally by donors Wellcome trust and others. **Reasons** The two Indian states that this trial was conducted in are two of the poorest states in India with high rates of illiteracy, poverty, and NMR (neonatal mortality rates). These are disproportionately higher than the rest of India. Poor access to health services.	**Location** Community-based **Length** Study conducted between 2005-2008	Quantitative Cluster randomised controlled trial. (outcomes for 19,030 births monitored)	**MCH outcomes** Neonatal mortality rate was 32% lower in intervention clusters during the 3 years and 45% in years 2 and 3. No significant change in maternal depression over 3 years but noted a 57% reduction in year 3. The most likely reason for lower mortality rate was the improved hygiene and care practices. **Wellbeing outcomes** The researchers hypothesized that the large decrease in maternal depression (year 3) might have come about due to increased problem-solving skills and improvements in social support. Researchers thought that the PWGs might have increased women's "critical consciousness"
17	Damtew, Z. A., Karim, A. M., Chekagn, C. T., Fesseha Zemichael, N., Yihun, B., Willey, B. A., & Betemariam, W. (2018)[53]	**Conceptual underpinning** Researchers mention the value of community health workers in other low resource settings. **Approach to CP** Instrumental	**Initiator** Ethiopian government introduced cadre of health workers and WDA. **Reasons** Ethiopian government addressed the shortfall in health workers by increasing	**Location** Community based. **Length** WDA began in 2011	Quantitative cross-sectional study (Household survey obtained data from 12,381 women)	**MCHquality care outcomes** This paper is concerned with an investigation of effectiveness of WDA comparing areas with high density WDA (up to 40 households) with those of low density (over 60 households). In the high-density WDA areas, in 6 of 13 indicators (including contraceptive prevalence rate, four or more ANC visits and hospital delivery), were significantly higher than in the low-density WDA areas. Indicating that strategies to train and deploy WDA shows great promise.

	Ethiopia Regions of Amhara, Oromia, Southern Nation Nationalities and Peoples' (SNNP)and Tigray	Contributions	spread of the Health Extension Workers (HEW) and the WDA. The reason was to network and discuss MCH issues, address geographical reach, equity, and improve maternal, neonatal and child health.			
18	Turan, J. M., Say, L., Güngör, A. K., Demarco, R., & Yazgan, S. (2003)[45] Turkey Fatih District, Istanbul,	**Conceptual underpinning** CP as a strategy in achieving "Health for All" **Approach to CP** In CDT Developmental empowerment, instrumental, In groups instrumental	**Initiator** Multidisciplinary research team at the Istanbul University Institute of Child Health **Reasons** Need for improvement in Turkey's perinatal health and quality of care. Following the failure of a hospital-based program an accessible community-based program, the Healthy Beginnings Program, with funding from a variety of sources, was introduced by hospital staff	**Location** Based in the Findikzade Education Park community centre. **Length** Commenced in 1997 and was still operating in 2003. External funding ceased in 2000.	Qualitative Program evaluation (pre- and post-knowledge tests with program participants; and home interviews conducted with course participants (n = 100) and a control group (n = 157) 2–3 months after the baby's birth)	**MCH promotion outcomes** Many pregnant women requested an education program for their husbands and their requests resulted in the development of a special program for expectant fathers. Free antenatal courses have started elsewhere, through train the trainers' courses. **Wellbeing outcomes** There was an increase in knowledge, skills, and decision making of the community design team (CDT). The program continued beyond funding as did the support networks between women.
19	Bolton, M., Moore, I., Ferreira, A., Day, C., & Bolton, D. (2016)[28] UK South London Lambert and Southwark	**Conceptual underpinnings** The 'broad-based' community –organizing model and methodology, deriving from the work of Saul Alinsky in Chicago. Putnam's concept of social capital **Approach to CP** Developmental Empowerment	**Initiator** Project was a collaboration of Citizen's UK Kings Health Partners (non-government) **Reasons** Importance of community engagement to health improvement (NICE) and Marmott's work. The aim was to evaluate a community-led intervention of social support to increase social capital, reduce stress and improve well-being in mothers who were pregnant	**Location** Community based but not in health – drew on relevant agencies and information as required by the group members. **Length** 2013	Quantitative Quasi-experimental design (Baseline and post intervention data collected from 15 mothers)	**Wellbeing outcomes** There were no detected changes in subjective well-being, but there were important reductions in distress on a standard self-report measure (GHQ-12). There were increases in social capital of a circumscribed kind (increase in volunteering and trust) associated with the project **Acceptability of MCH program** The programme was found to be feasible and acceptable to participating mothers and perceived by them to involve co-production and community control.

			and/or with infants aged 0 –2 years.			
20	Wong, M.L., Chen, P.Y.C. (1991)[46] Malaysia Remote part of Sarawak Berawan tribe Long Jagan	**Conceptual underpinnings** Conceptual base in development theory **Approach to CP** Developmental	**Initiator** The researchers commenced the project. **Reasons** Clear focus on women's development and empowerment as a way to improve health. Involving women in decision-making would assist in ownership of services and programs.	**Location** Training held at the head man's bilik **Length** Evaluation in 1991 one year after project commenced.	Participatory evaluation with some quantitative measures. No reporting of methodology rather descriptive of the program.	**MCH related service development outcomes** Rural women planned, organized, and implemented a self-help feeding program and kindergarten. 93% of mothers were involved in feeding program. Kindergarten attendance was 61%. Children looked cleaner. Increase in the proportion of households producing fruit and vegetables. **Wellbeing outcomes** Capability and self -reliance improved among women but were unquantifiable.
21	Rath, S., Nair, N., Tripathy, P. K., Barnett, S., Rath, S., Mahapatra, R., . . . Prost, A. (2010)[47] Eastern India Jharkhand (West Singhbhum and Saraikela Kharsawan) and Orissa (Keonjhar)/Rural	**Conceptual underpinning** Community participation. Freire's concept of development of 'critical consciousness' **Approach to CP** Developmental Empowerment	**Initiator** Researchers working with local women's groups and Ekjut **Reasons** Poor neonatal and maternal health outcomes. Although primary and community health centres were located in each of the clusters, villagers experienced multiple barriers to access, including physical distance, poor transport availability, and discrimination.	**Location** Community based. **Length** 2005-2008	Qualitative process evaluation (18 focus groups with community members, 15 focus groups with facilitators, 247 group discussions with group members, analysis of data collected by facilitators, document review)	**Process evaluation findings about community participation** Six key characteristics were found essential in community mobilisation. These are: (1) acceptability; (2) a participatory approach to the development of knowledge and skills; (3) community involvement beyond the groups; (4) a focus on marginalized communities; (5) the active recruitment of newly pregnant women into groups; (6) high population coverage.
22	Pant, P. R., Budhathoki, B., Ellis, M., Manandhar, D., Deave, T., & Mytton, J. (2015)[39] Nepal Makwanpur District Rural	**Conceptual underpinning** Community participation and social mobilisation for child injury prevention **Approach to CP** Instrumental	**Initiator** MIRA a Nepalese non-governmental research organisation. **Reasons** The programme was developed to cover a number of common unintentional injuries in children including falls, drowning, burns, poisoning, animal and road traffic injuries	**Location** Community based. **Length** Women's groups established for over a decade.	Mixed methods Program Evaluation (feedback forms completed by 30 community members; review of development of program resources, number of	**Program acceptability outcomes** Key findings emerging from feedback included strong support for a programme addressing child injury prevention from FCHVs, community leaders/ social workers and from women's group chairs. Particularly important in helping the reporting of children who had sustained injuries and in spreading the prevention messages. Feasible to collect parent-reported child injury outcomes through a community supported data collection system.

	Nepal Hatiya VDC				injuries reported)	
23	Nishiuchi, M. (1985)[54] Japan Suita (city)	**Conceptual underpinning** Not described **Approach to CP** Instrumental Empowerment and Developmental	**Initiator** Suita Health Centre and housewives of Suita **Reasons** Poor health of Suita children. Club chapter states its purpose as to promote infant healthcare, to prevent illness, to improve sanitary conditions of our community and thereby to lead healthy and cultural lives.	**Location** Primary health care service Initially set up as a childcare consultation service outside of the physical building of the health service. **Length** Set up in 1968 still going at time of publication with decreased numbers.	Mixed methods Narrative case study methodology not described	**MCH outcomes** As a result of consultation service and many other social factors– infant mortality rate declined . **MCH health promotion outcomes** Family planning – 1,000 education guidance sessions conducted, condoms delivered, and classes for pregnant women ongoing. Parasite control – Ascaris control campaign increased testing, detection and treatment. Trachoma campaign – increased examination and treatment Extermination of flies and mosquitoes – 5-year plan for insect eradication (not completely successful but viewed a success) TB – club asked by Suita city (council) to help with promoting community acceptance of TB detection – the club advocated for a more acceptable approach to implement this program.
24	Morrison, J., Osrin, D., Alcock, G., Azad, K., Bamjan, J., Budhathoki, B., . . . Houweling, T. A. J. (2019)[55] India, Nepal, Bangladesh, and Malawi. All the sites were rural	**Conceptual underpinning** Mechanisms of effective behaviour change model developed. The PLA approach was based on theory to transform society, address inequalities, local stakeholders should participate in identifying problems, planning, implementing and evaluating change. Theory of the mediating role of social-cognitive factors in the relationship between socioeconomic position and health behaviour. **Approach to CP** Developmental	**Initiator** Not reported **Reasons** Reduction in newborn mortality.	**Location** Community based. **Length** Unknown	Qualitative (42 focus groups and 15 interviews with women who attended groups and with women who had not attended, 12 key informant interviews and 5 focus groups with women's group facilitators and fieldworkers)	**MCH promotion outcomes** Women's groups addressed a knowledge deficit in poor and better-off women. Women learned from the facilitator and each other. Facilitators enabled inclusion of all socioeconomic strata, ensuring that strategies were low-cost and that discussions and advice were relevant. **Wellbeing outcomes** Groups provided a social support network that addressed some financial barriers to care and gave women the confidence to promote behaviour change (especially talking to in-laws). The social process of learning and action, which led to increased knowledge, confidence to act, and acceptability of recommended practices, was key to ensuring behaviour change across social strata. L**imitations** Researchers who were directly involved in the trials might have biased data collection and reporting. They felt that the disadvantages of this approach were outweighed by the advantages of having an experienced researcher who understood the intervention and process evaluation data

25	Morrison, J., Thapa, R., Sen, A., Neupane, R., Borghi, J., Tumbahangphe, K. M., . . . Costello, A. (2010)[41] Rural Nepal Makwanpur	**Conceptual underpinning** Broadly community participation **Approach to CP** Developmental Instrumental	**Initiator** MIRA a Nepalese non-governmental research organisation. **Reasons** Demand and supply-side factors influence health service use, yet advocacy has focused mainly on the supply of services. A randomized controlled trial was conducted in rural Makwanpur district, Nepal, to test the effect of a demand-side intervention – participatory women's groups – on neonatal mortality.	**Location** Community-based **Length** 2002-3	Qualitative (8 focus groups and 5 observations with women's groups, 7 groups discussion with fund users and 7 with potential fund users)	**MCH promotion outcomes** Management of funds increases women's self-efficacy and autonomy in health care decision-making. Funds were therefore a contributing component to the positive effect of the women's group intervention on neonatal mortality and care seeking. **Wellbeing outcomes** For the poorest women, funds may serve as a disincentive to joining groups, and therefore they were unable to benefit from the capacity building or empowerment that groups stimulate. **Limitations** The qualitative data were collected and analysed by advisors of the non-governmental organization implementing the trial, which may have biased the data collection. External researchers could not collect data because of the security situation and financial constraints.
26	Morrison, J., Thapa, R., Hartley, S., Osrin, D., Manandhar, M., Tumbahangphe, K., . . . Costello, A. (2010)[40] Rural Nepal Makwanpur	**Conceptual underpinning** The intervention borrowed from the literature on participatory approaches to community development, and the ideas of Paulo Freire, who argued that sustainable social change is only possible if teachers and learners engage in a dialogue, exchanging ideas and experiences **Approach to CP** Instrumental Developmental	**Initiator** Implemented by MIRA, a Nepalese non-governmental research organisation. **Reasons** Nepal's neonatal mortality rate is among the highest in Asia, at 33 per 1000 live births. Most births (81%) occur at home, and both community interventions and health service improvements are necessary to increase child survival	**Location** Community-based **Length** Trial 2002-3	Qualitative (Overall study RCT) (2 focus groups with women group members, 4 focus groups with non-members, 2 focus groups with mothers-in-law, 3 focus groups with men and 2 focus groups and 1 interview with health volunteers, observations of women's group meetings)	**MCH wellbeing outcomes from group experience** Four key mechanisms were identified: the groups learned, developed confidence, disseminated information within communities, and increased community capacity to take action. Provided support, a place for learning and sharing knowledge. The confidence of group members developed through their involvement with the intervention, and reticence decreased. Increased knowledge, confidence and encouragement from facilitators stimulated group members to disseminate information about maternal and neonatal health to other women. Planning and implementation of strategies enabled groups to mobilise community resources and strengthen social networks
27	Morrison, J., Tamang, S., Mesko, N., Osrin, D., Shrestha, B., Manandhar, M.,	**Conceptual underpinning** Community participation in planning and implementation leading to more cost-	**Initiator** The trial was implemented by MIRA a Nepali non-governmental organisation, and involved 24 Village	**Location** Community based. **Length**	Qualitative evaluation (Participant observation, reflections	**MCH Outcomes** Women's groups established mother and child health funds, producing clean home delivery kits and operating stretcher schemes. **MCH health promotion outcomes**

	. . . Costello, A. (2005)[2] Nepal/ Makwanpur district/Rural /Makwanpur hill and plain areas,	effective delivery of health care and increasing in service utilization. **Approach to CP** Developmental Instrumental – section on striking a balance	Development Committees in rural Makwanpur district. **Reasons** In response to plateauing of infant mortality rates	Most groups remained active after 30 months.	monthly reports and topic reports)	In response to the needs of the group, participatory health education was added to the Intervention. **Limitations** Group members faced difficulties when thinking about ways to tackle perinatal problems. Fatalism affected both the way people viewed themselves in relation to a problem, and also the power and capacity they believed themselves to have in overcoming it.
28	Lugo, N. R. (1996)[30] US/Orange County, Florida/The program was implemented in an inner-city area, a rural section of the county, and a quasi-suburban area	**Conceptual underpinning** Pablo Freire's work on empowerment education **Approach to CP** Empowerment Developmental	**Initiator** The Resource Sisters/Companeras Program organisers **Reasons** The Resource Sisters/Companeras Program was an effort to develop the conditions and a structure (with peer counsellors and peer support groups) to foster empowerment health education,	**Location** Meetings were held in clients' neighbourhoods, community rooms, health department sites, and trailer camps. **Length** 2 years at time of reporting	Mixed methods Program evaluation (Birth weights of participants n=1,117 compared with non-participants n=6,975, participation rates, description of project activities and content)	**MCH Outcomes** No statistically significant difference in birth weight between participants and non-participants. **Wellbeing outcomes** For participants, repeated participation suggest that the groups played a significant role in their lives and increased social cohesion. The Program appears to have had some success in capacity building, nurturing, and building upon the strengths, resources and problem-solving abilities already present in individuals. **Limitations** The requirements of well-documented individual assessments and case management (required by the funders the Healthy Start Program) ultimately drove the program, overshadowing the focus on groups and community development or community building.
29	Dongre, A.R., Deshmukh, P.R. & Garg, B.S (2009)[29] India Rural Wardha (District) 758km east from Mumbai	**Conceptual underpinning** Participatory approaches and culturally appropriate education materials were developed and piloted. **Approach to CP** Instrumental/ Developmental	**Initiator** Community Led Initiatives for Child Survival (CLICS) Program **Reasons** A Reduction in neonatal mortality as 1/2 neonatal deaths occur in first three days.	**Location** Village based. **Length** At least 3 years	Mixed methods Program evaluation (Baseline (n=404 and follow up n=393 Surveys, focus groups n=6)	**MCH promotion outcomes** Significant improvement, over 3 years, in mothers' knowledge of newborn danger signs, mother's health care seeking for sick newborns, and level of awareness of pregnant women. After three years, the proportion of mothers giving no treatment/home remedy for newborn danger signs declined significantly. **MCH service development** Local culturally appropriate health care plan and emergency transport plan developed. **Wellbeing outcomes** The capacity of the VCC members was strengthened during their monthly village-based meetings.
30	Dickinson, P. & Joe, T. (2010)[31]	**Conceptual underpinning** Social work practice framework of empowerment,	**Initiator** Te Waipuna Puawai, a community development initiative with board members)	**Location** Community based **Length** Since 2007	Qualitative Program evaluation	**MCH wellbeing outcomes** Increased social supports and networks for young mums, increased awareness of community support, increased awareness of self-value.

	New Zealand Metropolitan Auckland	cognitive, behavioural, and systems theory, strengths-based, a feminist/anti-oppressive approach, evidence-based practice, task-centred practice, holistic case work and community development. **Approach to CP** Developmental	**Reasons** The program aimed to give young mothers a space of their own to learn and grow, as well as providing information, advice, advocacy and support to create resourceful young mothers.		(7 review of course materials observations and feedback sessions with participants n=7)	
31	Tripathy et al (2016)[42] Rural Jharkhand and Odisha, eastern India	**Conceptual underpinning** PLA cycle explicitly mentioned and used as a theory of change. WHO recommendation on community mobilization through facilitated participatory learning and action cycles with women's groups for maternal and newborn health. **Approach to CP** Developmental Instrumental	**Initiator** The trial was led by Ekjut, a non-governmental organisation (NGO) working in Jharkhand and Odisha since 2003, in collaboration with the Institute for Global Health, University College London (UK). **Reasons** To test the effect of participatory women's groups facilitated by Accredited Social Health Activist (ASHA) on birth outcomes, including neonatal mortality. ASHAs might help with group meetings, and these might lead to improvements mother's practices and reduction in neonatal mortality.	**Location** Community based. **Length** Although the intervention was originally planned for 24 months, it lasted for 31 months (Sept 1, 2010, to March 30, 2013), because the groups chose to hold additional meetings in phase 3.	Quantitative cluster randomised controlled trial. The intention-to-treat analysis for the primary outcome included 7219 births during 24 months of intervention (Jan 1, 2011, to Dec 31, 2012).	**MCH Outcomes** During the period Jan 1, 2011, to Dec 31, 2012 the neonatal mortality rate was 30 per 1000 livebirths in the intervention group and 44 per 1000 livebirths in the control group. These findings corresponded to a 31% reduction in neonatal mortality when data were adjusted for clustering and stratification by district (OR 0·69, 95% CI 0·53–0·89). The effect was more pronounced when results were adjusted for baseline differences in neonatal mortality (adjusted OR 0·54, 95% CI 0·36–0·80). The intra-cluster correlation coefficient for neonatal mortality was 0·00282.
32	Morrison, Tumbahangphe, Sen, Gram, et al. (2020)[32] Hills district of Makwanpur, central Nepal	**Conceptual underpinning** Community participation through PLA **Approach to CP** Developmental Instrumental	**Initiator** Mandated by government to be facilitated by Female Community Health Volunteers (FCHV) **Reasons** To raise awareness about health issues. To make	**Location:** Community-based **Length** Trial 2010-2012	Cluster randomised control trial. 21 intervention Village Development Committees with 22 control clusters	Cluster randomised control trial. 21 intervention Village Development Committees with 22 control clusters 39% of deliveries in institutions. No statistically significant difference in treatment arms between institutional delivery or attendance by trained health worker. **Limitations of method.** Unable to maintain status of control clusters as they received similar interventions provided by Community Based

			services more accountable. Part of wider strategy to engage citizens and communities in health.		FCHV group meeting attendees 14.3% (896) in intervention areas over 4.7% (353) in control.	Organisations (mandated by Makwanpur District Development Committee). Previous project meant 78% of women's groups in control active at baseline (so not true control). Therefore, absence of a true counterfactual.
33	Ronaasen, Steenkamp, Williams, Finnemore, & Feeley (2020)[33] Nelson Mandela Bay, Eastern Cape, South Africa	**Conceptual underpinning** Developmental model (Van Huysssten 2014) interactions amongst support group participants in the context of their social surroundings may lead to the development of skills, knowledge and increased capacity. PAR **Approach to CP** Developmental	**Initiator** Collaboration between Nelson Mandela University, UNICEF and NGO **Reason** Need to increase breast feeding, reduce social isolation due to HIV associated stigma and connect families to health care services.	**Location** Early childhood development centre **Length** Unclear	**Study design** Process evaluation through the qualitative data collected from 2 focus group discussions with 30 facilitators and four with 34 group members	**Socioemotional wellbeing outcomes** Increase in mutual connection between women gained by attending groups. Health and social support gained by sharing knowledge. Improved confidence in parenting ability
34	Sharma, S., Mehra, D., Akhtar, F., & Mehra, S. (2020)[56] Banda and Kaushambi districts in Uttar Pradesh	**Conceptual underpinning** Theory of change used as was community engagement with peer educators. **Approach to CP** Instrumental	**Initiator** Lund University Sweden and MAMTA Health Institute for Mother and Child Delhi **Reasons** Increase access to health services in order to reduce the maternal mortality rate and improve child health.	**Location** Community based. **Length** 30-month period April 2013 until September 2015	Mixed method evaluation. Non experimental post-test analysis of the project group using mixed method rapid assessment surveys with 476 women. Management information systems data for 37,324 women and qualitative assessment with focus groups.	**MCH health promotion** A perceived increase in mother's knowledge about MCH and about accessing health services. The gap in the service utilization rates between the women from non-marginalized and marginalized families seemed to have decreased after the intervention revealed in interviews. **Wellbeing outcomes** Women appeared to be financially stronger and more independent. Perceived change in equity norms in communities

35	Colbourn, T.,Nambiar, B., Bondo, A., et al (2013) [34] Three rural districts of Malawi	**Conceptual underpinning:** Use of PLA cycles to build confidence and empowerment **Approach to CP** Developmental	**Initiator** UCL Institute for Global Health, and NGO MaiKhanda **Reasons** Malawi is offtrack to meet Millennium Development Goal 5 (a three-quarters reduction in maternal mortality between 1990 and 2015).	**Location** Community-based Mid 2007 to 2010	Two-by-two factorial cluster randomized controlled trial. Interventions; Quality improvement in MCH services and women's group intervention. Primary outcomes were maternal, perinatal and neonatal mortality.	**MCH Quality improvement** In rural Malawi a combined facility improvement (supply side) and women's group community intervention (demand side) reduced neonatal mortality by 22%, and a women's group intervention alone reduced perinatal mortality by 16%, compared with control areas.